

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: Patient at 32 weeks gestation has a medium risk for preeclampsia, indicated by elevated systolic blood pressure (142 mmHg) and proteinuria (1), though no severe symptoms are reported. There is also a medium risk for macrosomia due to high pre-pregnancy BMI and elevated fasting glucose. Alerting the doctor for review of preeclampsia risk.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: Patient ID: TEST_MED_MED, Gestational Age: 32 weeks. Preeclampsia risk tier: Medium (Score: 0.64). Macrosomia risk tier: Medium (Score: 0.59). Vitals: Systolic BP 142 mmHg, Diastolic BP 79 mmHg, Proteinuria dipstick 1. No severe symptoms (headache, vision changes, upper abdominal pain, swelling of face/hands) reported. Fasting glucose: 99 mg/dL. Pre-pregnancy BMI: 42.4. Recommend clinical evaluation for preeclampsia risk based on elevated BP and proteinuria, as per ACOG guidance on hypertensive disorders of pregnancy.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

